

Otafuku

The ports on her body gone by the time she woke up. On first seeing the scars the ports left behind, she didn't know what to think. Cycled between the theory of being hurt by others and of hurting herself, and didn't see either option as very plausible. Despite that, anything was plausible under the auspices of the droughts. When she herself finally came to peace with the droughts, her understanding grew and she found herself at the base of the mountain: she was using the droughts to heal others.

It made sense from where she was sitting. During the droughts, patients would present to her as having the symptoms of her day job patients: heart problems. Namely, tachy and bradyarrhythmias. Mostly tachy, which is to say pathologically fast heart rates caused by *something* within the heart of the patient. The solution, in waking life at least, was to identify the mechanism or mechanisms and address them by selectively damaging the tissue to halt whatever circuit within the heart there was. This all was, as she learned, very different during a drought. Instead of sedating the patient or damaging *the patient's* heart, Dr. Otafuku would herself get on the patient table in the cath lab for the procedure and allow the patient in question to speak freely and from the heart. More often than not, Dr. Otafuku would not listen to the content of the patient's speech. Rather she would pay close attention to something deeper, something inexpressible in the way they spoke. Her team at the hospital would gain access via her groin if it was the first case for the drought and conduct the procedure somewhat normally as compared to the day-to-day version, but this was all done with the patient there, in the room, talking and sometimes crying. They would watch Dr. Otafuku grimace and murmur and whelp in pain. It was an upsetting thing to see someone go through so much for you. Some of the patients didn't live the most healthy lifestyles, and so for them, this process was harrowing; it opened their eyes in a way that the day-to-day version of the procedures couldn't. In fact, the normal procedures typically *closed* their eyes, caused them to forget and generally fumble through the haze of the healing process. Our Dr. Otafuku though, was somewhat of a specialist. As in, no-one-else-could-do-this type of specialist. Other cardiac electrophysiologists had their own gifts or tricks or what have you, but none quite met the symbolic level that Dr. Otafuku's reached. Because the procedure was done in sterile fashion, even the patient was there at the table in a surgical gown, mask, and bouffant or cap. The patients who preferred to use radiation for the procedure would be wearing heavy lead, that was weighted dynamically: the more they felt, the less the lead weighed; the less they felt... The drought tended to get through to them, let's just say.

What didn't initially make sense to Dr. Otafuku was this process of hearing without listening. As soon as she put language to it though, it immediately became clear that the process she was facilitating, along with her patient, stood in direct opposition to much of the training she had received in her life, specifically regarding bedside manner and collecting patient history. How could you hear

without listening? This question tore her up on the backend of the early droughts, it undermined a lot of what she believed in. Particularly because the skill set in question was something she had to work at a little longer than her peers had. It took time for her to pick up on subtle cues and clues. Frequently she would reduce her earliest patients in her career down to what was on paper, not out of spite or anything so malevolent, but simply because their narrative was shaky, clumsy, and usually error-prone. Patients lied but tests didn't. Yet there she was in just this last drought, listening to a patient drone on and on about the silliest of things. Or so she was told, because of course *she* wasn't listening. Her staff didn't run on fumes; rather, they ran on sighs and goodnatured, time-killing humor. They generally got antsy throughout these cases, though, because they just didn't have "it": they had no idea when the patient was close to having the arrhythmia in question solved and sorted. And so, apart from the mechanical work of assisting the patient with the procedure and access and getting catheters up into Dr. Otafuku's heart, the staff had nothing to do and it all kind of just felt like a big fancy waiting room to them. And, honestly, it wasn't such a bad thing. Boring yes, but also good fun. For when the staff forgot themselves and joined in the motion of the drought, they quite enjoyed the procedures that Dr. Otafuku ran.

The best days were when the patient communed with Otafuku and the decision was made to call in the ultrasound retinue. Much excitement filled the air when this came about because the ultrasound team was not just man and machine the way it was during the waking hours. Instead, ultrasound entailed two species: human and bat. Usually megabats, but sometimes micro if a patient (and so therefore Otafuku herself) was of small stature. Bats worked with other specialties too, of course, but the ultrasound techs had the most charisma and fun. Anesthesia, for example, had a close bond with vampire bats (as one might expect).